TALON-2 interim analysis: recommendation to the DMC

Closed session, 9 December 2026. Prepared by Dr Rachel Savage, independent statistician, KUCTU. Data cut-off 17 November 2026 (the 165th BICR-confirmed event); database snapshot 1 December 2026.

Recommendation

Continue TALON-2 to its final analysis at 330 events as planned (Schedule 2, band C). The conditional power of the final analysis is 100 per cent to the nearest 10 per cent (above 99 per cent in the projection), well above the band's 80 per cent.

Interim data

All 450 patients were randomised by the cut. 87 of the 165 events are on placebo and 78 on ravolimab. The stratified hazard ratio is 0.91 (95 per cent CI 0.67 to 1.24), stratified log-rank z 0.58. Carried into the remaining follow-up as a single ratio, that estimate gives a conditional power of about 5 per cent, which would fall in band A.

The single ratio averages two different periods. Through the week-12 assessment ravolimab patients progressed slightly faster than placebo patients (rate ratio 1.34); from the week-18 assessment onward they progressed at 0.38 times the placebo rate. Most of the interim events fell in the first period, because most patients have short follow-up; 200 of the 267 patients still at risk at the cut are past the week-12 assessment, so most of the events still to come fall in the second.

Assessment	Placebo: events / patient-months	Ravolimab: events / patient-months	Rate ratio
Week 6	27 / 413	37 / 413	1.37
Week 12	19 / 199	24 / 194	1.29
Week 18	17 / 140	8 / 140	0.47
Week 24	6 / 98	4 / 105	0.62
Week 32 onward	18 / 73	5 / 97	0.21

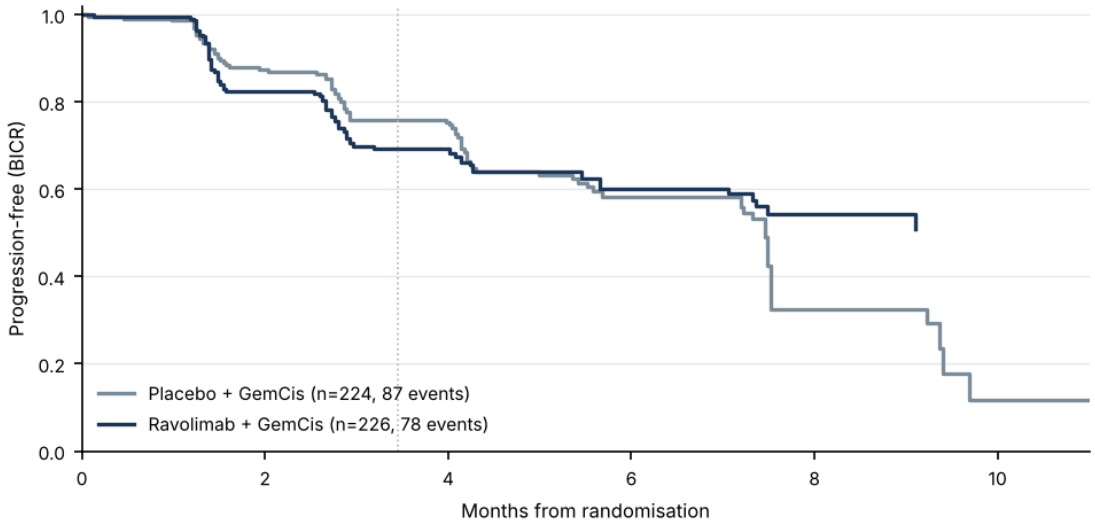

Figure 1. Kaplan-Meier estimates of BICR progression-free survival by arm at the interim cut. The dotted line marks the gap between the week-12 and week-18 assessments.

Conditional power

As the charter defines it (section 4.1): each patient still at risk at the cut was followed forward from the last adequate assessment on the arm's progression rate before and after the week-12/week-18 gap, with withdrawal at the observed rate, until the 330th event, and the final stratified log-rank test was recomputed in 20,000 simulated continuations; it rejects in 19,992 of them. Under the sample-size hazard ratio, which the charter excludes, the figure would be about 54 per cent.

Safety and exposure

Serious adverse events (pharmacovigilance database), liver test elevations after the first dose and exposure are in talon2_interim_tables.csv. Hepatic events are more frequent on ravolimab, as expected for the class; none of the tables calls for a safety recommendation.